GBV EDUCATIONAL CONTENT LIBRARY / CHAPTER 07

Emotional, Psychological and Verbal Abuse

Recognizing harmful patterns, responding safely, and supporting people experiencing non-physical abuse

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Educational and safety notice

This chapter is for education, not diagnosis, investigation, legal advice, or a substitute for professional GBV response. A person may recognize patterns of abuse without using a particular label. Do not use the exercises to pressure anyone to disclose experiences. If there is immediate danger, seek a safer location if possible and contact appropriate emergency or protection services. Referral numbers should be rechecked at the time of use.

Learning objectives

- Explain emotional, psychological and verbal abuse, including how they overlap with coercive control.
- Identify warning patterns while distinguishing abuse from ordinary disagreements.
- Understand possible effects on wellbeing, choices, relationships and everyday functioning.
- Practice nonjudgmental, survivor-centred listening and safety-aware help-seeking.
- Apply concepts to fictional Rwanda-relevant situations without requiring personal disclosure.

How to use this chapter

Read sections in order or select those most relevant to a learning session. The scenarios are fictional, and no person should be asked to identify a real survivor in group discussion. Trainers should explain confidentiality and safeguarding boundaries before discussion and offer the option to pass on any activity. References marked [1]–[7] are listed at the end.

01 Understanding non-physical abuse

Emotional and psychological abuse refers to behavior intended to frighten, degrade, manipulate, isolate, or control someone, or behavior that has these effects within a pattern of coercion. Verbal abuse can include repeated insults, threats, humiliation, or degrading language. The terms overlap: verbal abuse describes one way harm may be expressed, while psychological abuse can also involve nonverbal threats, isolation, surveillance, and restrictions on freedom. [1, 2, 3]

Abuse is not defined solely by whether a person shouts. A calm threat, deliberate humiliation in public, or repeated manipulation can be frightening and controlling. Conversely, a single argument, disappointment or respectfully expressed criticism does not automatically establish an abusive pattern. Context, impact, power, intent, repetition, fear and control all matter. One serious threat can also be dangerous even without a long history.

Common patterns

- Humiliation and belittling: mocking abilities, appearance, identity or worth; insulting someone in front of others.
- Threats and intimidation: threatening harm, expulsion, abandonment, reputational damage, or taking children away to force compliance.
- Isolation: discouraging or forbidding contact with friends, family, healthcare workers or trusted supporters.
- Monitoring and control: demanding explanations for everyday movements, controlling messages, or insisting on constant access to a device.
- Manipulation: repeatedly distorting events, denying agreed facts, blaming the harmed person for the abuse, or using affection and withdrawal to secure obedience.
- Undermining independence: blocking access to learning, employment, money, care or basic decisions.

When is it gender-based?

Abuse can be gender-based when it is linked to gender, gender expectations, discrimination or unequal gender-related power. People of any gender can be harmed. The fact that someone experiences harsh words does not on its own establish GBV; the surrounding context is essential. [1]

02 Verbal abuse versus respectful conflict

Conflict happens in many relationships. People can disagree, express anger and set firm boundaries without using abuse. Healthy disagreements leave room for each person to speak, say no, seek support and make independent choices. Abuse may involve fear, repeated degradation, punishment for disagreement or behavior designed to control another person.

Compare the examples

- Respectful: “I feel worried when plans change without notice. Can we discuss a better way to communicate?”

- Concerning: “You are useless. Nobody else would put up with you, and you will regret talking to your friends.”
- Respectful: A supervisor explains performance standards privately and invites questions.
- Abusive: A supervisor repeatedly mocks an employee, threatens their job unless they accept inappropriate demands and blocks complaints.

The goal is not to label every difficult interaction. It is to notice patterns of fear, coercion and reduced freedom, and to seek appropriate assessment or support when needed.

03 Coercive control: understanding the pattern

Coercive control is a pattern of behavior that restricts another person’s autonomy through fear, threats, monitoring, isolation or dependency. Several seemingly small acts may accumulate into a serious loss of freedom. A person may begin avoiding ordinary activities simply to prevent retaliation. [2, 3]

For example, a partner may gradually demand passwords, decide who may visit, discourage employment and threaten humiliation if challenged. None of these acts needs to involve a physical injury to create risk. Coercive control can occur alongside physical or sexual violence, and escalation may be unpredictable.

04 Manipulation and reality-distorting behavior

Manipulation uses pressure, guilt, fear, misleading information or emotional leverage to influence decisions. Some abusive behavior involves consistently denying events, changing explanations, or making someone question their memory and judgment. The popular term “gaslighting” is sometimes used for such patterns, but it should not replace a careful description of actual actions.

What this may look like

- A person denies having sent threatening messages even when they were received, then accuses the recipient of being irrational.
- A partner demands secrecy about mistreatment and says the survivor is responsible for “destroying the family” if they seek help.
- A manager alternates threats with praise to pressure someone into accepting inappropriate demands.

A safe response focuses on observable behavior and its effects. Do not insist that someone confront an abusive person, gather evidence on a monitored device, or win an argument about what “really happened”.

05 Threats, humiliation and intimidation

Threats may be direct (“I will hurt you”) or indirect (“You will see what happens if you tell anyone”). A threat to remove children, end access to money, disclose private information or damage someone’s reputation can be used to exert control. Humiliation and name-calling may erode confidence and lead to withdrawal, while intimidation may make a person feel unsafe even without physical contact. [2]

Threats involving weapons, serious violence, stalking, confinement, or harm to children require urgent attention. A survivor’s own assessment of danger deserves serious consideration. No public handbook can accurately predict an individual’s risk.

06 Emotional abuse in different settings

Intimate partnerships and families

Repeated insults, isolation, monitoring, threats, blaming and restrictions on freedom can occur in marriages, dating relationships and other family contexts. Family reputation, financial interdependence or child-care responsibilities may complicate choices. The person harmed should not be blamed for staying, leaving, returning, or delaying disclosure.

Schools and universities

An adult or peer may use humiliation, threats, rumor-spreading or promises of academic advantage to control a learner. An unequal position of authority can make refusal difficult. Education providers need safe reporting channels that protect learners from retaliation, particularly where minors are involved.

Workplaces

Abusive behavior may include repeated public degradation, threats to employment, discriminatory insults, isolation from colleagues or retaliation for raising concerns. Not every unfair workplace decision is necessarily GBV; assess the behavior and gender-related context. Workers may need independent reporting and support options.

Digital environments

Emotional abuse can continue through excessive messaging, threats, impersonation, public humiliation, monitoring or coercive access to accounts. Digital risks may increase if a person tries to change settings on a device the other person controls.

07 Possible effects on survivors

Non-physical abuse can be associated with fear, anxiety, low confidence, sleep disruption, social isolation, difficulties concentrating and reduced participation at school or work. Some people experience depression or trauma-related symptoms, but no specific reaction proves that abuse occurred. Responses differ, and a person may function outwardly while experiencing serious harm. [1, 2]

Psychological symptoms are not a diagnosis. A qualified clinician should evaluate persistent or serious mental health symptoms, and medical or psychosocial assistance should be offered according to the person’s needs and wishes.

08 Why leaving or reporting may be difficult

People experiencing abuse may face threats, economic dependence, concerns for children, social stigma, disability-related barriers, insecure housing, immigration concerns, emotional attachment or fear that no one will believe them. Leaving or reporting may temporarily increase risk in some situations. The safest next action is individual and should not be prescribed by an untrained person. [2, 4]

Avoid asking “Why didn’t you leave?” A better question is: “What would feel most helpful and safe for you right now?”

09 Supporting someone who discloses abuse

WHO describes the LIVES approach for first-line support: Listen, Inquire about needs, Validate, Enhance safety and Support. In non-clinical settings, this is best used as an orientation to compassionate responses, not as permission to perform clinical risk assessments or investigations. [4, 5]

- Listen: give time, avoid interruptions and do not demand a complete account.
- Inquire: ask what the person needs now rather than collecting unnecessary details.
- Validate: acknowledge the person’s experience; avoid blame or disbelief.
- Enhance safety: ask whether it is safe to continue talking and help identify qualified support; do not impose a safety plan.
- Support: offer verified referral options and help with access only with appropriate consent, subject to safeguarding obligations.

Words that may help

“Thank you for telling me.” “You deserve to be treated with respect.” “You do not have to tell me more than you want to.” “Would you like information about confidential support options and any limits to confidentiality?”

10 Confidentiality, privacy and safeguarding

Ask whether it is safe to call, text or leave messages. Do not assume that a phone or internet account is private. Avoid storing names, details or screenshots without a legitimate reason, a secure process and informed consent. Do not share a survivor’s story with family, community leaders or social media contacts. Explain any relevant mandatory reporting, child-protection or imminent-danger exceptions before inviting sensitive disclosure.

For children, involve trained safeguarding personnel using age-appropriate procedures and applicable local rules. Do not promise absolute secrecy. Where there is a clear immediate threat, prioritise urgent protection and professional assistance.

11 Safety-aware help-seeking

- If it is safe to do so, seek a private conversation with a trusted professional or specialist GBV service.
- If the person is being monitored, consider whether contacting a service from a safer device or location is feasible; avoid making abrupt technology changes that could increase danger.
- A survivor may choose to discuss possible safe contacts, essential needs and emergency options with a trained provider.
- Do not encourage direct confrontation, retaliation, or mandatory couples counseling in the presence of coercive control; these may increase risk.
- If there is immediate danger, obtain emergency assistance through appropriate local channels.

12 Rwanda-specific support and referral

Rwanda's Ministry of Gender and Family Promotion describes Isange One Stop Centres as coordinated entry points providing healthcare, psychosocial support, justice-related services and referral to shelter when needed. UNHCR Rwanda also provides information about service access. [6, 7]

Published contacts — verify before distribution

- Rwanda Investigation Bureau (RIB): 116 / 3512 (MIGEPROF published numbers).
- Gender Monitoring Office: 5798 (MIGEPROF published number).
- MIGEPROF: 9059 (MIGEPROF published number).
- Isange One Stop Centre: 3029 (UNHCR Rwanda published number).
- National Child Development Agency: 711 (MIGEPROF published number).

Important: UNHCR cautions that approaching an Isange Centre may result in the incident being reported to law-enforcement authorities. Explain this possibility before a survivor shares identifiable information and confirm local confidentiality and reporting processes with qualified personnel. These contacts do not guarantee an immediate answer or suitability in every situation. [7]

What should an educator do?

Offer the person the option to contact a verified service privately and discuss transport, accessibility or communication barriers. Do not make a formal report on an adult survivor's behalf without understanding the relevant law, safety implications and consent, except where applicable legal or safeguarding duties require action.

13 Scenario 1 — A university student and constant monitoring

Fictional case: A university student in Kigali receives dozens of messages each day from a dating partner asking for location updates. When the student attends a study group, the partner calls repeatedly, insults the student and threatens to post private messages online. The student begins withdrawing from classmates to avoid conflict.

Learning discussion: What patterns are concerning? Repeated monitoring, humiliation, threats and isolation suggest coercive control. What would be unsafe? Instructing the student to confront the partner immediately or change phone settings without assessing the risk. What might help? Private, nonjudgmental support and information about trusted services.

14 Scenario 2 — Humiliation at work

Fictional case: A supervisor repeatedly tells a female employee she is incapable of leadership because of her gender, insults her during meetings and threatens her promotion if she complains. A colleague wants to help but is worried about retaliation.

Learning discussion: The gender-based insults, threats and unequal power are important. The colleague could ask privately whether support is welcome, avoid circulating the story, and offer information about independent workplace reporting or specialist services. The colleague should not promise that reporting will be risk-free.

15 Scenario 3 — Family pressure and isolation

Fictional case: A married adult in Musanze is regularly told that speaking to relatives is a betrayal. Their partner controls access to transport money and threatens to remove contact with the children if the adult seeks advice.

Learning discussion: Multiple behaviors—social isolation, threats and financial restriction—may work together. The affected adult’s priorities and safety should guide any referral. Pressuring immediate separation could increase danger.

16 Scenario 4 — Digital humiliation

Fictional case: A former partner sends insulting messages, creates fake accounts and threatens to publish private information. Friends suggest responding publicly with screenshots, but the person feels unsafe.

Learning discussion: Online harassment and threats can be forms of emotional abuse. Public retaliation may intensify harassment. Appropriate options depend on safety, available support and local procedures. A specialist can help consider documentation and reporting without exposing sensitive information unnecessarily.

17 Scenario 5 — A child who hears repeated threats

Fictional case: A young adolescent hears one caregiver threaten another and repeatedly witnesses humiliation at home. The child becomes worried about going home and has difficulty concentrating at school.

Learning discussion: The child’s distress is important even without direct physical assault. A responsible adult should follow established safeguarding procedures, avoid asking the child to investigate or mediate, and seek support from appropriate child protection professionals.

18 Five guided learning activities

Activity A — Recognize the pattern (individual or group; 10 minutes)

Read a fictional statement: “If you visit your friends, I will make sure no one trusts you again.” Identify the behavior (threat and isolation pressure), the possible impact (fear and withdrawal), and a safer support response (private, nonjudgmental assistance). Do not ask participants to share personal examples.

Activity B — Respectful or controlling? (10 minutes)

Compare: “Can we agree on how to communicate when plans change?” versus “Give me your passwords or I will punish you.” Discuss which response leaves room for choice, and how threats change the situation.

Activity C — Safe listening role-play (15 minutes)

One participant reads a fictional disclosure, another practices a supportive response, and an observer checks for listening, validation, choice and privacy. Do not role-play an aggressive perpetrator or reenact violence. Any participant may opt out.

Activity D — Support pathway mapping (15 minutes)

Without naming survivors, identify a school counselor, health facility, Isange Centre or other verified local resource and document accessibility, confidentiality limits and the safest way to make contact. Confirm contacts before using them in real referrals.

Activity E — Digital privacy thought experiment (10 minutes)

Consider a fictional person whose messages may be monitored. Discuss why “just block the person” or “change every password now” may be unsafe without considering the circumstances. Suggest contacting a specialist from a safe channel when possible.

19 Reflection without disclosure

- What is the difference between criticism and a pattern of degradation?
- How might threats affect a person’s freedom even without physical injury?
- Which supportive phrases respect the person’s choices?
- What should be checked before referring someone to a local service?

A learner may reflect privately or choose not to answer. Collecting identifiable disclosures is not necessary for this exercise.

20 Knowledge assessment — ten questions

1. Can abuse occur without physical violence?

Suggested answer: Yes. Psychological control, humiliation, threats and isolation can cause harm.

2. Is every disagreement emotional abuse?

Suggested answer: No. Context, fear, patterns, coercion and impact matter; one severe threat can nevertheless be dangerous.

3. What is coercive control?

Suggested answer: A pattern that restricts autonomy through threats, monitoring, isolation, intimidation or dependency.

4. Why might a person remain in an abusive situation?

Suggested answer: Safety threats, financial constraints, children, attachment, stigma and many other barriers may be involved.

5. Is telling a survivor to confront the person harming them good advice?

Suggested answer: Not automatically. Confrontation can increase risk; safety and choice come first.

6. What should a supporter do first after a disclosure?

Suggested answer: Listen without judgment, ask what support is wanted and check whether the conversation is safe.

7. Should you promise total confidentiality?

Suggested answer: No. Explain any legal, child safeguarding or imminent-danger exceptions.

8. Can digital monitoring be part of abuse?

Suggested answer: Yes. Coercive monitoring, threats and unauthorized access can be part of controlling behavior.

9. Is it appropriate to diagnose PTSD based on someone’s story?

Suggested answer: No. Provide support and referral; a qualified professional assesses mental health diagnoses.

10. What is a Rwanda-specific service pathway?

Suggested answer: A verified Isange One Stop Centre or other appropriate service, with reporting and confidentiality implications explained.

21 Glossary

Emotional abuse: Behavior that undermines emotional wellbeing through degradation, threats, manipulation or other harmful treatment.

Psychological abuse: Abusive behavior that creates fear, distress or coercion, including nonverbal control.

Verbal abuse: Insults, degrading language, threats or humiliating speech used to harm or control.

Coercive control: A pattern of behaviors that limits choices and independence.

Intimidation: Actions or communication used to create fear.

Survivor-centred: An approach prioritizing safety, dignity, informed choices and individual needs.

Safeguarding: Procedures to protect people, especially children and at-risk people, from harm.

Referral: Connecting a person to an appropriate qualified service according to need and safety.

22 References and further reading

[1] World Health Organization (2026). Violence against women.

https://www.who.int/news-room/fact-sheets/detail/violence-against-women

[2] World Health Organization (2012). Understanding and addressing violence against women: Intimate partner violence.

https://iris.who.int/bitstream/10665/77432/1/WHO_RHR_12.36_eng.pdf

[3] Australian Institute of Health and Welfare (2026). Coercive control.

https://www.aihw.gov.au/family-domestic-and-sexual-violence/understanding-fdsv/coercive-control

[4] World Health Organization (2014). Health care for women subjected to intimate partner violence or sexual violence: A clinical handbook.

https://www.who.int/publications/i/item/WHO-RHR-14.26

[5] World Health Organization (2019). Caring for women subjected to violence: A curriculum for training health-care providers.

https://www.who.int/publications-detail-redirect/9789241517102

[6] Rwanda Ministry of Gender and Family Promotion. Family promotion and child protection: GBV prevention and Isange One Stop Centres.

https://www.migeprof.gov.rw/family-promotion-and-child-right-protection

[7] UNHCR Rwanda. Gender-based violence: Services and referrals.

https://help.unhcr.org/rwanda/services/sgbv/

23 Publication and review checklist

- GBV practitioner or safeguarding specialist reviews safety advice and survivor-centred language.
- Qualified Rwanda legal/safeguarding reviewer verifies confidentiality and reporting obligations.
- Referral coordinator verifies phone numbers, location, eligibility, hours and possible law-enforcement reporting.
- Kinyarwanda and French translations undergo independent review for nuance and safety.
- All activities are tested for accessibility, informed choice and potential distress.
- CMS assigns version history, next review date, content owner and approval status.

Editorial status

Evidence-informed educational draft only. No claim of professional clinical, legal or safeguarding approval has been made. Review the entire chapter before publishing or adapting it for an AI chatbot or public awareness campaign.

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